

PREFACE

Why This Book Exists — And Why the Starting Point Cannot Be a Blank Page

American healthcare is not waiting for transformation to begin. It is already transforming — haltingly, unevenly, and under financial pressure that is forcing changes no stakeholder would have chosen voluntarily. Hospitals are closing maternity wards. Rural emergency departments are converting to lower-acuity models or shuttering entirely. Insurance premiums are consuming an ever-larger share of household income. Primary care physicians are leaving independent practice for hospital employment or leaving medicine entirely. The workforce is exhausted. The administrative burden is crushing. And the populations bearing the worst consequences — rural communities, low-income families, aging Americans — are the ones with the least political power to accelerate change.

This book is written in that reality, not above it.

Every proposal in these pages must be understood as constrained by where the American healthcare system actually is — not where a reformer might wish it to be. There is no blank slate. There is no greenfield on which a theoretically optimal system can be designed from scratch. There is instead a \$4.5 trillion system with millions of employees, thousands of facilities, hundreds of entrenched contractual relationships, fifty state regulatory architectures, multiple federal program structures, and decades of accumulated institutional behavior. Transformation, in this context, does not mean replacing the system. It means systematically altering its fundamental incentives, relationships, and capabilities — from within — in a sequence and at a pace that the system's existing constraints make feasible.

Understanding this is the first and most important premise of the six-pillar framework this book presents. It is also the single most common source of reform failure: the gap between what a policy analysis recommends in isolation and what an embedded, historically conditioned system can actually execute. Reforms designed as if the system were a blank canvas — as if hospitals were not already financially fragile, as if physicians were not already overwhelmed, as if state agencies had surplus organizational capacity, as if federal programs were immediately flexible — produce technically correct recommendations that collapse on contact with operational reality.

The Inheritance That Reform Must Work With

To build on the American healthcare system is to inherit everything it has accumulated. That inheritance includes genuine achievements that no serious transformation agenda would discard: the most sophisticated biomedical research enterprise in the world; a network of academic medical centers that generates the clinical knowledge base on which global medicine depends; near-universal electronic health record adoption following the HITECH Act of 2009; a partially functional insurance exchange architecture created by the Affordable Care Act; an aging but established primary care infrastructure anchored by patient-centered medical home programs in several states; decades of quality measurement work through HEDIS, HCAHPS, and CMS value-based purchasing; and a growing, if incomplete, value-based care contracting ecosystem.

That same inheritance also includes structural features that actively resist transformation. The fee-for-service payment model — which accounts for the overwhelming majority of American healthcare transactions — rewards volume over value, complexity over prevention, and individual procedures over coordinated population health management. The commercial insurance market's pricing architecture depends on dominant hospital systems charging commercial payers two to four times Medicare rates — a pricing structure that cross-subsidizes below-cost Medicare and Medicaid payments but makes premiums increasingly unaffordable for individuals and employers. The administrative infrastructure supporting fee-for-service billing — prior authorization, utilization review, claims coding, appeals — consumes an estimated 25 to 35 percent of total healthcare spending and has no analogue in any comparably advanced healthcare system.

Hospital systems have built their financial models, their facility footprints, their workforce configurations, and their governance structures around this inherited architecture. So have physician practices, insurance companies, pharmacy benefit managers, device manufacturers, and the thousands of ancillary organizations that constitute the healthcare industry's supply chain. Transformation that ignores this institutional conditioning will generate resistance that has nothing to do with ideology and everything to do with survival.

The six-pillar framework is designed to work within this inheritance — to identify the intervention points, the sequencing logic, and the enabling conditions that make structural change achievable in a non-greenfield system. That is a harder analytical problem than designing an optimal system from scratch. It is also the only problem worth solving.

What the Historical Record Actually Teaches

The United States has been attempting healthcare transformation, in one form or another, for forty years. The accumulated evidence from those attempts is the single most important input into any serious transformation framework — not because the evidence is clean or the lessons are simple, but because the pattern of failure is consistent enough to be analytically instructive.

The voluntary payment reform era — from the early managed care experiments of the 1980s through the ACO proliferation of the 2010s — produced one clear and repeatedly confirmed finding: voluntary reform achieves marginal results among willing participants while leaving the rest of the system unchanged. The most instructive version of this lesson is OneCare Vermont, the state's Accountable Care Organization, which operated for nearly a decade as a voluntary model and produced modest results while the hospitals most responsible for Vermont's cost crisis declined to participate. OneCare is not a Vermont anomaly. It is the national ACO experience in miniature. Of the CMMI's fifty-plus experimental models since 2010, the vast majority produced no statistically significant reduction in spending. Most were discontinued. The few that showed durable results shared a common feature: some mechanism that made participation consequential, not optional.

The technology investment era — from the HITECH Act's EHR subsidies to the current generation of population health platforms — produced a second clear and repeatedly confirmed finding: technology infrastructure without aligned incentives produces tools that no one has a financial reason to use effectively. Vermont's own VITL health information exchange exists and is technically functional. It is not widely used. The most sophisticated interoperability platform in the world cannot change clinical behavior if the payment model still rewards volume over value.

The clinical quality improvement era — three decades of HEDIS measurement, pay-for-performance programs, and evidence-based guideline development — produced genuine quality improvements in measurable domains. It did not reduce cost growth. Clinical excellence is rewarded by the fee-for-service system only to the extent that it generates billable encounters. Prevention, coordination, and early intervention — the clinical functions most likely to reduce total cost — are structurally disadvantaged in a volume-based payment environment.

The equity improvement era — waves of disparity measurement, diversity initiatives, and community health investment — produced detailed documentation of the problem without systematic reduction in racial, geographic, or socioeconomic health gaps. Equity cannot be bolt-on. It cannot be addressed by equity-specific programs layered on top of a system that distributes its fundamental resources — care access, clinical quality, workforce — along lines of privilege. Equity requires structural redesign, not supplemental programming.

The transformation framework this book presents takes all four of these historical lessons seriously. It does not propose a fifth round of voluntary payment reform. It does not propose technology investment without payment alignment. It does not propose clinical quality improvement without structural change to what quality generates financially. And it does not propose equity programs that leave the underlying resource distribution unchanged.

Vermont as the Teaching Case — And Its Limits

Vermont runs through this book as its primary teaching case. This requires honest explanation of both why Vermont was chosen and what it cannot represent.

Vermont is in this book because it is the most advanced real-world experiment in simultaneous multi-pillar healthcare transformation currently underway in the United States. Acts 167 and 68 of the Vermont legislature — passed in 2022 and 2025, respectively — together constitute the most comprehensive statutory mandate for structural healthcare reform any state has enacted. They create mandatory reference-based pricing. They require global hospital budgets. They mandate a statewide strategic plan with measurable outcome commitments. They direct the Agency of Human Services to restructure itself from a program-administration organization into a

population health system operator. They are backed by the Green Mountain Care Board, Vermont's unique all-payer hospital rate-setting authority — the institutional architecture that makes mandatory payment reform operationally feasible.

Vermont is also transparent in ways that make analysis possible. The Oliver Wyman Act 167 statewide analysis, the GMCB's monthly hospital performance data, the AHS transformation reports, and the 14-hospital transformation plans submitted in January 2026 constitute a documentary record of a system in real-time structural stress that no other state can match. Vermont's reform is happening in public, with detailed data, in front of an engaged legislature, a watchful press, and a small enough civic community that the consequences of decisions are traceable and attributable.

And Vermont is early. Early in its demographic aging. Early in its hospital financial fragility. Early in its recognition that voluntary reform cannot produce structural change. The forces producing Vermont's crisis — an aging population shifting toward Medicare and Medicaid, a shrinking commercially insured base undermining the cross-subsidy that sustains rural hospitals, fee-for-service payment rewarding volume in a system that needs to reduce it — are present in every American state on a lag of ten to twenty years.

But Vermont's limits as a teaching case must be stated with equal clarity. Vermont is the second-smallest state by population in the country, with 647,000 residents and an entirely non-profit hospital system, no for-profit hospital chains, a strong civic culture of public-sector engagement, and a regulatory apparatus with no direct analogue in most states. The Green Mountain Care Board's mandatory rate-setting authority — the institutional foundation on which Vermont's mandatory payment reform rests — took decades to build and depends on enabling legislation and political conditions that other states will need to create deliberately if they do not already possess them.

The portfolio approach this book develops is designed to be transferable — not by copying Vermont's specific policy instruments, but by applying the same analytical logic to the specific institutional inheritance, political environment, and starting conditions of any state or system. What transfers is the framework: the identification of the six pillars, the sequencing logic derived from dependency analysis, the failure cascade predictions, and the portfolio management discipline that holds the transformation together across time. What requires adaptation is every specific policy choice.

A Portfolio Approach, Not a Plan

This book frames healthcare transformation as a portfolio management problem. The distinction from conventional reform planning is not semantic.

A reform plan assumes a defined end state and a linear path to reaching it. It assumes that the policy environment, the financial context, the technological landscape, and the organizational capabilities of the implementing institutions will remain stable enough that the plan written today will describe what happens tomorrow. In healthcare transformation, none of these assumptions hold. Federal policy changes. Hospital systems consolidate or fail. Technology evolves faster than governance can follow. Workforce shortages worsen or ease unexpectedly. Political coalitions shift.

A portfolio approach accepts this instability as its operating condition. It manages a set of interdependent investments — across the six pillars — with explicit attention to sequencing, dependencies, risk, and the conditions under which the portfolio must be adjusted. It does not commit to a single predetermined path. It commits to a direction, a set of foundational investments that must be made regardless of how the environment shifts, and a decision logic for how to adapt when the environment does shift.

The six pillars of the framework — Policy, Technology, Economics, Clinical, Equity, and Operations — are the portfolio's asset classes. The sequencing logic developed in Chapter 2 is the portfolio's investment horizon framework. The dependency analysis in Chapter 1 is the portfolio's risk management architecture. Vermont is the portfolio's most detailed case study of what it looks like to manage all six asset classes simultaneously in a real system under real financial pressure.

A portfolio approach is also honest about failure modes. Not all investments in the portfolio will succeed. Some pilots will produce evidence that a particular clinical model does not work in a given context. Some technology

investments will underperform. Some payment reform experiments will generate unexpected consequences. The portfolio approach treats these as informative failures — inputs into the adaptation logic — rather than catastrophic deviations from the plan. This is how any serious long-term transformation must be managed.

A Note on Contested Terrain

Healthcare transformation is politically contested, and this book engages that contestation honestly. Reference-based pricing is opposed by hospital systems that depend on commercial rate premiums. Global budgets are feared by hospitals and physician groups that worry about access constraints. AHS restructuring is resisted by programs that do not want to be reorganized. The equity investments this framework requires are contested by those who believe resource allocation should be determined by market mechanisms. These are not irrational positions. They reflect genuine interests, legitimate concerns, and in some cases, serious analytical objections.

This book does not adjudicate these contests as political matters. It does present the evidence on what has worked and what has not, the analytical logic of the six-pillar framework, and the sequencing rationale derived from the dependency structure. Where the evidence is strong, this book says so. Where it is contested or uncertain, it says that too. The goal is not to end the political debate but to elevate it — to give every participant a more rigorous analytical foundation for the positions they hold and the decisions they make.

How to Read This Book

For healthcare policy professionals:

Read the Preface and Introduction in full, then Chapters 1 and 2 to establish the six-pillar framework and sequencing logic. Appendix A provides the complete Vermont system portrait. Chapter 16 (AHS Restructuring) is the most direct bridge between policy mandate and organizational design. Chapter 15 projects the national implications.

For healthcare executives and administrators:

Read the Introduction for the overarching argument, then proceed to the chapters most relevant to your immediate decisions. Chapter 6 (Economics) for value-based contract navigation. Chapters 12 and 13 (Technology) for data infrastructure decisions. Chapters 7 and 8 (Operations) for transformation planning methodology. Return to Chapters 1 and 2 for the integrating logic.

For Vermont practitioners:

Begin with Appendix A (Vermont System Portrait), then read Chapters 2 and 3 (Execution Sequence and Policy Pillar) as the framework most directly grounded in Vermont's current situation. Chapter 16 provides the operational framework for the AHS restructuring work. The implementation timeline tables throughout Part II serve as a working reference guide. The Vermont in Practice sidebars are written specifically for you.

For students and researchers:

Read the book in chapter order. Chapter 1 establishes the framework. Chapters 3 through 13 develop each pillar with full academic grounding before the Vermont application. Chapters 14 through 16 cover the advisory infrastructure, the national forecast, and the AHS restructuring roadmap. The Key Concepts glossary at the end of each chapter and the source notes constitute the academic apparatus.

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INTRODUCTION

The Transformation Imperative: Building on What Exists

I. Starting Where the System Actually Is

On September 18, 2024, the Green Mountain Care Board of Vermont convened a public meeting at which Oliver Wyman Healthcare and Life Sciences Group presented the findings of a year-long, \$1 million statewide hospital systems analysis commissioned under Act 167 of 2022. The room was full. The findings were not surprising to anyone who had been paying attention — but they were stated with precision and comprehensiveness that made them impossible to dismiss.

Nine of Vermont's fourteen hospitals were reporting operating losses in 2023, with the worst reaching negative 8.9%. If current trends continued, thirteen of the fourteen would be reporting losses by 2028. The cumulative five-year system deficit was projected between \$700 million and \$2.4 billion. Average premiums for silver exchange plans had increased 108% in six years, while median household income had grown 22%. Out-of-pocket maximums had more than doubled. Vermont's working-age population was declining toward a projected 13% reduction by 2040, while its over-65 population was growing toward 30% of the state's total — a demographic shift that progressively converts patients from commercially insured to Medicare and Medicaid, both of which reimburse hospitals below cost.

The Oliver Wyman consultants offered three structural prescriptions: build housing and fix transportation; pay hospitals with reference-based pricing and move to global budgets; move all care possible out of hospitals. These were not incremental recommendations. They were diagnoses of a system approaching structural failure.

Vermont is not an outlier. It is a preview.

The structural forces producing Vermont's crisis are present in every American state — demographic aging, hospital financial fragility, commercial cross-subsidy unsustainability, voluntary payment reform failure, workforce exhaustion under administrative burden. Vermont faces them earlier and more acutely because of its demographics and economic structure. The states of the Northeast, the rural Midwest, and eventually the national system will arrive at Vermont's current condition on a lag of ten to twenty years. What Vermont is forced to solve now, the rest of the country will be forced to solve later.

But — and this is the critical premise of this book's analytical approach — the urgency of Vermont's crisis does not alter the fundamental constraint under which any transformation must operate: the system that needs to be transformed is the system that must do the transforming. There is no external authority that will replace Vermont's hospitals with better-designed ones, no federal program that will install a new payment architecture overnight, no technological solution that will arrive fully formed and universally adopted. Transformation must be built from within the existing system, working with its actual capabilities, its real institutional incentives, and its genuine organizational constraints.

This Introduction develops the analytical framework for doing exactly that.

II. The System's Starting Conditions — What Must Be Inherited

Before proposing any transformation agenda, the analyst must conduct an honest inventory of the system as it exists. This inventory has two parts: what the system does well that transformation must preserve, and what the system does that actively prevents transformation.

What Must Be Preserved

The American healthcare system's genuine achievements are not incidental to transformation — they are its foundation. Academic medical centers at the frontier of biomedical research. Near-universal EHR adoption

creating a digital record of clinical care. Decades of quality measurement infrastructure through HEDIS, HCAHPS, and CMS programs. A partially functional exchange architecture enabling insurance portability. An established, if underfunded, primary care infrastructure in most communities. A growing value-based care contracting ecosystem with real performance data and lessons learned. And — specifically in states like Vermont — regulatory architectures with proven authority to set rates and enforce accountability.

Transformation that destroys these assets while building new ones wastes capital, alienates workforce, and creates political opposition that derails reform. The portfolio approach this book develops is designed to build on existing assets, not replace them.

What Prevents Transformation

The system's structural barriers to transformation are equally important to name precisely:

- The fee-for-service payment model rewards volume, not value. Every primary care visit that prevents a hospitalization generates no revenue for the prevention. Every hospital that succeeds in reducing its avoidable admission rate reduces its own income. The clinical logic and the financial logic point in opposite directions.
- The commercial cross-subsidy architecture requires dominant hospitals to charge commercial payers 200% to 400% of Medicare rates to cover the losses from below-cost Medicare and Medicaid reimbursement. This pricing structure produces premiums that are increasingly unaffordable while being structurally necessary for hospital solvency — a contradiction that no single-pillar intervention can resolve.
- Voluntary participation structures allow the highest-cost, highest-revenue actors to opt out of reform. The ACO experience from 2010 to 2025 — fifty-plus CMMI models, the overwhelming majority discontinued without achieving statistically significant savings — is the empirical record of what voluntary payment reform achieves at scale.
- The administrative burden of fee-for-service billing — prior authorization, utilization review, claims coding, appeals — consumes an estimated 25 to 35% of total healthcare spending and absorbs workforce capacity that should go to patient care. It also creates organizational dependency: entire administrative departments, revenue cycle management companies, and consulting firms are structured around the billing complexity that transformation would eliminate.
- Technology investment without payment alignment produces digital infrastructure that no one has a financial reason to use effectively. Vermont's VITL health information exchange and the national EHR ecosystem are the proof cases.

The transformation agenda must address all five of these structural barriers simultaneously. That is the central analytical argument of the six-pillar framework: these barriers are interdependent, and interventions that address any one in isolation run into the constraints created by the others.

III. The Sequencing Question — Why Order Is Not Optional

The most consequential analytical question in any transformation agenda is not what to do. The evidence on what needs to happen — pay differently, redesign care delivery, build data infrastructure, address social determinants, ensure equity, execute operationally — has been reasonably settled for decades. The consequential question is in what order to do it.

The dependency structure of the six pillars generates a specific, defensible answer to this question. Some investments cannot produce their intended results until enabling investments are already in place. Getting the sequence wrong is not merely inefficient — it produces structural failures that are expensive to reverse and politically difficult to explain.

Chapter 2 develops the execution sequence in detail, grounded in Vermont's OneCare failure as the primary negative proof case. But the sequencing logic can be summarized here, because it shapes every subsequent chapter:

The Six-Pillar Execution Sequence — Core Logic
Stage 1 POLICY: Mandatory architecture and transformation capital must precede all else. Voluntary reform is not structural reform. The enabling legislation, regulatory authority, and federal agreements that convert aspirational policy into binding obligation are the preconditions for every subsequent investment. Policy also secures the transformation capital — federal grant awards, federal-state agreements — that funds the technology build.
Stage 2 TECHNOLOGY: Data infrastructure must be built before financial risk is assumed. The OneCare failure is the proof: managing global budgets without integrated, timely clinical and claims data is the 'managing blind' failure mode. Technology is the operating system on which every other pillar's management functions depend. It must be operational before the payment model goes live.
Stage 3 ECONOMICS: Payment incentives deploy on a system that can already see itself. Reference-based pricing, global budgets, and total cost of care accountability change the financial logic that shapes organizational and clinical behavior. They must go live after the technology substrate is in place, not before. Payment architecture design happens in parallel with the technology build; payment management begins only when data infrastructure is ready.
Stage 4 CLINICAL: Care redesign on aligned incentives. Primary care transformation, behavioral health integration, care management at scale — the clinical programs where the economic return on the entire investment is realized. Clinical redesign without payment alignment reverts when the incentives revert. It comes fourth because it requires stages 1-3 to already be functional.
Stage 5 EQUITY: Systematic audit and calibration — plus social risk adjustment embedded in Stage 3. Equity is a constraining dependency through every earlier stage, not a final add-on. Social risk adjustment in payment design (Stage 3), culturally competent care models (Stage 4), and targeted clinical deployment in underserved communities (Stage 4) are equity functions that precede Stage 5. What Stage 5 adds is population-level measurement of whether earlier-stage equity commitments have produced equitable outcomes, and corrective action informed by real data.
Stage 6 OPERATIONS: Closing the administrative cost gap and translating strategy into implemented programs. Vermont's critical-access hospitals spend \$2,730 per discharge on administration against a national benchmark of \$1,427. This operational gap is the primary source of projected direct savings from shared services and simplification. Operations is last not because it is least important, but because operational redesign requires the payment environment it is optimizing for to already be operational.

This sequence is not a rigid waterfall. Payment architecture design proceeds in parallel with the technology build. Operational planning begins before technology is complete. Equity analysis runs throughout. What the sequence specifies is what cannot go live before its enabling conditions are in place — and what the failure mode looks like when that discipline is abandoned.

IV. The Six Pillars — An Integrated System, Not a Checklist

The six-pillar framework is the analytical architecture that organizes this book. Each pillar represents a domain of necessary investment; the fifteen dependency relationships between them represent the mechanisms through which each pillar's effectiveness depends on the others.

Pillar	Structural Role — What It Produces When Functioning
Policy	The mandatory architecture. Converts aspirational reform into binding requirements. Without mandatory authority, the highest-cost actors opt out and voluntary reform achieves marginal results. Vermont's Acts 167 and 68, the AHEAD Model State Agreement, and the mandatory participation requirements are the Policy pillar's operative products.
Technology	The data substrate. Makes population health management, value-based contract execution, equity measurement, and strategic planning analytically feasible. You cannot manage what you cannot measure. Vermont's VHCURES, VTTL/VHIE, and the AHS-GMCB analytics vendor are Technology pillar investments.
Economics	The incentive architecture. Determines whether organizations have a financial reason to behave differently. Payment reform does not change clinical behavior directly — it changes the financial logic that shapes clinical decisions over time. Vermont's reference-based pricing and global hospital budgets are Economics pillar instruments.
Clinical	The mechanism of change. Payment reform changes incentives; clinical redesign changes behavior. The financial case for preventive investment only materializes when clinical programs actually deliver the utilization reduction that produces savings. Vermont's Blueprint for Health (5.8:1 ROI), CCBHC expansion, and behavioral health integration are Clinical pillar investments.
Equity	The cross-cutting constraint and accountability lens. Not a separate program — a dimension applied to every decision in every other pillar. Transformation that improves averages while widening disparities has failed by the framework's own standards.
Operations	The execution layer. Translates statutory mandates, payment models, data platforms, and clinical programs into organizational reality. Vermont's \$1,303 per-discharge administrative cost gap above the national benchmark is the Operations pillar's quantified opportunity.

The most common failure mode in healthcare transformation is single-pillar thinking: addressing payment reform without clinical redesign, or deploying technology without payment alignment, or pursuing equity goals without structural SDOH investment. Each intervention, pursued in isolation, runs into the constraints created by the pillars it ignores. This is not an analytical abstraction. It is the empirical record of thirty years of healthcare reform attempts in the United States.

Vermont's reform agenda is instructive precisely because Acts 167 and 68, the AHEAD Model, and the Rural Health Transformation Program together address all six pillars — imperfectly, as any real-world policy effort does, but with a comprehensiveness that distinguishes Vermont's approach from the single-pillar interventions that produced the reform record described above.

V. Vermont's Specific Starting Conditions — The System Vermont Must Transform

For the Vermont-specific analysis that follows throughout this book to be analytically useful, readers need an accurate picture of the system Vermont began with. This is not a portrait of a blank canvas. It is a portrait of a system under stress, with specific institutional strengths, specific financial vulnerabilities, and a specific regulatory architecture that makes some things possible in Vermont that are not possible elsewhere.

The Regulatory Asset: The Green Mountain Care Board

Vermont's most important structural advantage in healthcare transformation is institutional, not financial. The Green Mountain Care Board — established in 2011 under Act 48 — has had authority to set hospital budgets (not merely review them) since fiscal year 2013. This is not a rate review program. It is mandatory rate-setting authority over all fourteen Vermont hospitals. When GMCB sets a budget, hospitals must operate within it. When hospitals exceed their budgets, GMCB can reduce future commercial rate approvals as enforcement. When the legislature mandates global budgets, GMCB is the institutional mechanism that makes the mandate operational.

This regulatory architecture is the Policy pillar's foundation. Other states attempting payment reform without a comparable regulatory body are attempting the Economics pillar without the Policy pillar structure that gives payment reform enforcement authority. The OneCare failure was partly a symptom of this gap: the ACO could set targets, but it could not compel the hospitals responsible for most of Vermont's costs to participate in accountability for those costs.

The GMCB's institutional significance extends beyond Vermont. It represents the policy design that other states must build if they want to pursue mandatory payment reform. Vermont did not acquire this institution quickly or easily. It took decades of political investment. States beginning that investment now are building the institutional precondition for the payment reform that will become necessary when their own fiscal crises arrive.

The Financial Crisis: What the Numbers Require

Vermont's hospital system is in acute financial distress that makes the sequencing of transformation non-negotiable rather than merely desirable. Nine of fourteen hospitals reported operating losses in FY2023. Oliver Wyman projects thirteen of fourteen in losses by 2028 absent structural intervention. The cumulative five-year deficit is projected between \$700 million and \$2.4 billion.

This crisis has a specific structure that shapes the transformation agenda. Critical Access Hospitals — the smallest, most rural institutions serving communities with the fewest alternatives — face average operating profit of \$938 per adjusted discharge against a \$2,784 national benchmark. The University of Vermont Medical Center, Vermont's largest hospital with approximately 50% of market share, has an administrative cost per adjusted discharge of \$3,826 against a national benchmark of \$1,427. These are two different problems requiring two different interventions: the CAH problem is a revenue adequacy and care model problem requiring both payment reform and clinical redesign; the UVMHC problem is an administrative cost problem requiring operational transformation.

The premium crisis overlays and drives the hospital financial crisis. Vermont's average silver plan premium rose 108% between 2018 and 2024 — from \$456 to \$948 per month — while median household income grew 22%. The mechanism is direct: Vermont hospitals charge commercial payers 250 to 300% of Medicare rates on average, with UVMHC charging approximately 417% of Medicare for outpatient services in 2022. These prices translate directly into premiums. Mandatory reference-based pricing — Act 68's FY2027 mandate — is the Economics pillar instrument designed to break this mechanism.

The Demographic Constraint

Vermont's demographic trajectory is the foundational constraint on the transformation's timeline. Vermont's over-65 population will reach over 30% by 2040 — up from 21.7% in 2020, a 57% increase in the elderly share of the population. The working-age population will decline 13% from 367,000 to approximately 318,000. Total population remains essentially flat at approximately 620,000.

This demographic shift does two things simultaneously: it increases demand for complex, high-cost care while reducing the commercial insurance base that cross-subsidizes below-cost Medicare and Medicaid payments. The cross-subsidy model that has sustained Vermont's rural hospital system is becoming structurally impossible to maintain — not in a distant future scenario, but on the demographic trajectory Vermont is already on. Every year without structural transformation is a year in which the financial gap widens and the organizational capacity to execute transformation narrows.

The Technology Gap

Vermont's data infrastructure is more developed than most states', and less developed than the transformation it is attempting requires. VHCURES — Vermont's all-payer claims database — provides foundational longitudinal data but arrives with a two-year lag in most available analyses, making real-time population health management impossible. VTTL — Vermont's health information exchange — exists and is technically functional but is not widely used by community hospitals. Vermont's clinical data and administrative data are not integrated in a way that supports the attribution, risk stratification, and total cost of care measurement that global budgets require.

The AHS-GMCB analytics vendor procurement — awarded to McKinsey in early 2026 — is the Technology pillar investment designed to close this gap before the AHEAD Model's global budgets go live in January 2027. The timing matters: technology must precede financial risk assumption, not follow it. Vermont's launch of the AHEAD Model in January 2027 before the analytics platform is fully deployed represents the most significant sequencing risk in the current transformation architecture.

VI. The Pillar Sequencing — Validating or Revising the Book's Proposed Order

The original framework proposed a sequencing of six pillars: Policy, Economics, Operations, Clinical, Equity, Technology. The analysis developed throughout this book — grounded in the OneCare failure autopsy, the Vermont financial crisis, the Technology-Economics dependency relationship, and the historical record of reform failures — leads to a revised sequencing: Policy, Technology, Economics, Clinical, Equity, Operations.

The two key changes from the original order deserve explicit justification:

Why Technology Moves from Sixth to Second

The original framework treated Technology as the sixth pillar — a capability to be built after the foundational policy, payment, and clinical work was done. The OneCare failure demonstrates why this sequencing is incorrect and why its inversion produces the 'managing blind' failure mode.

Technology is not a layer applied on top of a functioning payment reform. It is the operating system on which payment reform's management functions depend. A global budget without integrated, timely data is a financial accountability commitment with no operational mechanism for accountability. Vermont tried this with OneCare, and the consequences were predictable: care managers working from 60-to-90-day-old claims data identified patients who had already been hospitalized. Intervention opportunities had already passed. Costs had already been incurred. The financial accountability was real; the management capability was absent.

The correct sequencing is: secure the policy mandate and the capital to fund the technology build (Stage 1: Policy); build the technology infrastructure while simultaneously designing the payment architecture (Stage 2: Technology); deploy the payment model on a system that can already see itself (Stage 3: Economics). This is the sequence AHEAD's design attempts to enforce — the analytics vendor procurement precedes the January 2027 global budget launch — but as noted above, the timing of the vendor deployment creates a sequencing risk that Vermont must manage carefully.

Why Operations Moves from Third to Sixth

The original framework placed Operations third — recognizing its importance as the execution layer but perhaps overestimating how quickly it could be built independent of the payment environment it was executing within. The revised sequencing places Operations sixth not because it is least important, but because operational redesign must be calibrated to the payment environment it will operate in.

Vermont's administrative cost gap — \$2,730 per discharge against a \$1,427 national benchmark for rural hospitals — is the quantified opportunity. But an administrative redesign optimized for fee-for-service billing is a different redesign than one optimized for global budget management. Building shared services infrastructure, consolidating administrative functions, and redesigning revenue cycle operations before the payment model is live risks optimizing for the world that transformation is replacing rather than the world it is creating.

Operations must be planned throughout — the organizational capacity for AHS restructuring, the 14-hospital transformation planning process, the RHRC engagement — but the heavy administrative cost reduction work is most productively executed after the payment model is operational and the clinical models are defined. The Operations pillar's execution is both an ongoing enabling function and a Stage 6 optimization opportunity.

VII. A Portfolio in Motion — Vermont's Transformation Architecture as of April 2026

Vermont's transformation is not a future plan. It is a portfolio of active investments, with several already deployed, several in implementation, and several approaching critical deadlines. The following table maps Vermont's current transformation architecture against the six-pillar sequence, providing the reader with a real-time snapshot of where Vermont stands as of this book's writing.

Stage / Pillar	Status as of April 2026	Next Critical Milestone
1. Policy	Complete (foundational): Acts 167, 51, 68 enacted. AHEAD State Agreement signed January 2025. \$195M RHT Program awarded December 2025. GMCB mandatory authority established.	December 2028: Statewide Strategic Plan delivered to legislature
2. Technology	In progress (critical path): AHS-GMCB analytics vendor procured (McKinsey, early 2026). VHCURES operational; two-year data lag remains. VITL functional; utilization incomplete. Vermont CIN under development.	Analytics platform must be operational before January 2027 AHEAD launch
3. Economics	In progress: Act 68 FY2026 commercial rate benchmark (-1%) in effect. GMCB developing RBP methodology. AHEAD Medicaid global budgets operational January 2026.	October 2026: RBP mandatory for commercial payers. January 2027: AHEAD Medicare global budgets live.
4. Clinical	Active: Blueprint for Health operational; 7 CCBHCs certified by mid-2026. 14-hospital transformation plans submitted January 2026. Primary Care AHEAD design in progress.	July 2026: 5 new CCBHC certifications. January 2027: Primary Care AHEAD replaces Blueprint Medicare payments.
5. Equity	Early stage: Act 68 equity requirements embedded. HSA-level geographic monitoring initiated. SDOH screening through Blueprint operational. Race/ethnicity data gaps in VHCURES remain.	Strategic Plan (2028) must include equity measurement framework with HSA-level disaggregation.
6. Operations	Planning stage: AHS restructuring underway. 14-hospital plans in draft. RHRC engaged. PMO establishment recommended but not yet complete. \$1,303/discharge administrative cost gap unaddressed.	AHS organizational restructuring; HSA Coordinator roles; Division of Planning and Effectiveness establishment.

This is not a complete or entirely orderly execution. Vermont's transformation is proceeding under financial pressure, organizational constraints, and political conditions that no analytical framework can fully control. The Technology pillar's analytics deployment is behind the ideal sequencing relative to the Economics pillar's AHEAD launch. The Operations pillar's PMO infrastructure is not yet complete. The Equity pillar's data gaps constrain measurement before they constrain outcomes.

These are not failures of the framework. They are the conditions under which real transformations happen — messy, constrained, and requiring continuous adaptation. The portfolio approach is designed to manage this

messiness, not eliminate it. The framework provides the analytical architecture for understanding where the risks are, which gaps have the most consequential cascade effects, and where marginal investment has the highest leverage.

VIII. What This Book Can and Cannot Do

Intellectual honesty requires stating clearly what this book offers and what it does not.

This book provides the analytical framework for understanding what is happening in American healthcare transformation and why — the six-pillar architecture, the sequencing logic derived from dependency analysis, the failure cascade predictions, and the Vermont evidence that tests the theoretical structure against empirical reality. It supplies the evidence base for evaluating what has worked and what has not, across thirty years of reform attempts at the national level and in Vermont's specific experience. It develops the conceptual vocabulary for communicating clearly across the disciplines — clinical, financial, policy, technological, operational — that transformation requires.

This book does not predict whether Vermont's transformation will succeed. The statutory mandates are in place. The capital is flowing. The organizational machinery is in motion. The outcome depends on thousands of decisions that will be made by hospital executives, primary care physicians, community health workers, legislators, regulators, and patients over the next five to ten years. The framework provides the analytical basis for those decisions; it cannot make them.

This book does not substitute for the operational expertise required to implement transformation in a specific organizational context. The frameworks are accurate and the evidence base is solid, but implementing a global budget requires actuarial expertise, regulatory knowledge, and operational experience that no book can provide. Advisory services, technical assistance organizations, and experienced practitioners are the bridge between this analytical framework and specific organizational decisions.

This book does not resolve the political contests that surround healthcare transformation. Reference-based pricing, global budgets, mandatory participation, AHS restructuring — all are contested by legitimate interests with real stakes. Those contests will be resolved by political processes that this book can inform but cannot replace.

What this book can do is give every participant in those processes — policymaker, administrator, clinician, analyst, advocate — a more rigorous analytical foundation for the positions they hold and the decisions they make. Healthcare transformation is too consequential, and too difficult, for anything less.

The Central Argument of This Book in Five Sentences

American healthcare transformation is not a blank-canvas design problem. It is a portfolio management problem constrained by a \$4.5 trillion system's institutional history, financial structure, and organizational capabilities.

The six pillars of transformation — Policy, Technology, Economics, Clinical, Equity, and Operations — are interdependent, and transformation fails when any pillar is addressed in isolation.

The order in which the pillars are activated matters as much as the content of the investments: Policy must establish mandatory architecture and transformation capital before anything else; Technology must precede Economics as management; Clinical transformation requires aligned payment incentives; Equity must constrain every pillar's design; Operations executes last but plans throughout.

Vermont is the most advanced real-world experiment in simultaneous six-pillar transformation currently underway in the United States — not because Vermont is large, but because it is early, transparent, and

operating under financial pressure severe enough to make sequencing discipline mandatory rather than merely desirable.

The analytical framework Vermont's experience validates is transferable to every state and health system that will face Vermont's conditions on a ten-to-twenty-year lag — which is to say, every state and health system in the United States.

— *Proceed to Chapter 1: The Six-Pillar Framework*

Sources

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